# 3.16 Thoracic Surgery (Lobectomy and Pneumonectomy)

| Assessment | Nursing Diagnosis | Plan / Expected Outcome | Nursing Interventions | Evaluation |
|---|---|---|---|---|
| **Subjective**<br>• "It hurts to breathe."<br>**Objective**<br>• Breathing shallow<br>• Less air entry at the base<br>• $SpO_2$ 92% | **ACTUAL**<br>**Ineffective breathing pattern** related to incisional pain and reduced lung volume as evidenced by shallow breathing and low $SpO_2$. | $SpO_2$ 94% or more and using the incentive spirometer. | • Give analgesia regularly (epidural or PCA as ordered).<br>• Every hour, encourage deep breathing and incentive spirometry.<br>• Get him moving early.<br>• Position as ordered (after pneumonectomy, do not turn him fully onto the non-operated side). | $SpO_2$ on target: **goal met**. |
| **Subjective**<br>• Puts the pain at 8/10<br>**Objective**<br>• Guarding<br>• Moves the arm on the operated side less | **ACTUAL**<br>**Acute pain** related to thoracotomy incision as evidenced by pain score and guarding. | Pain 3/10 or less and doing his arm exercises. | • Check pain regularly.<br>• Teach shoulder ROM exercises.<br>• Have him hold a pillow over the incision when he coughs. | Pain controlled, arm moving fully: **goal met**. |
| **Subjective**<br>• *No specific complaint*<br>**Objective**<br>• Major thoracic surgery<br>• Chest drains | **RISK**<br>**Risk for bleeding** as evidenced by risk factors: major surgery and chest drains. | Vital signs stable and drain output within the expected limits. | • In the first hours, record chest drain output every hour.<br>• Follow vital signs and Hb.<br>• Report any drainage that is too much. | No excessive bleeding: **goal met**. |